Strategic UX/UI Architecture and Figma Component Blueprint for a Unified Healthcare Case Management System

Executive Context and Platform Rationalization

The modernization of healthcare operational technology requires transitioning from fragmented, disparate point solutions to cohesive, unified platforms. An analysis of the current operational architecture for community health organizations—specifically those managing complex Medicare and Medicaid populations—reveals profound inefficiencies stemming from a reliance on an unintegrated constellation of applications. The legacy stack under review, which includes Salesforce for case management, Tellescope for patient customer relationship management (CRM), Healthie for electronic health records (EHR), Candid/Bridge for revenue cycle management (RCM), and Five9 for contact center operations, generates significant cognitive friction for end-users.[1] This fragmentation leads to manual data handoffs, double-entry burdens, increased clinical error rates, and a distinct lack of real-time visibility into member engagement and clinical outcomes.[1]

The objective of this comprehensive design blueprint is to architect the User Experience (UX) and User Interface (UI) components required for a Unified Internal Platform. This platform functions as a Care and Case Management System (CMS), designed to serve as a single source of truth for all member interactions, clinical documentation, event operations, and contact center workflows.[1] The design strategy outlined herein systematically categorizes the functional requirements from Care Delivery to Compliance, translating complex business logic into actionable Figma design system components. Furthermore, this report proactively addresses the architectural UI requirements for Shared Medical Appointments (SMA), a future-state module requiring specialized group event rostering and batch billing interfaces.[1]

Foundational Design System and Figma Architecture

To ensure scalability, consistency, and rapid iteration across a platform encompassing over fifty distinct functional requirements, the UX strategy must be rooted in a robust, component-based design system built natively within Figma. This system must adhere to stringent accessibility standards while supporting the complex, data-heavy workflows characteristic of clinical and operational environments.

The design system must employ an atomic design methodology, organizing elements from basic inputs (atoms) to complex dashboards (organisms). By utilizing Figma's advanced features—such as Auto Layout 5.0, nested components, interactive variants, and comprehensive variable modes—the design team can ensure that interfaces adapt fluidly across desktop and mobile breakpoints.[3] The Figma architecture must prioritize reusable

component sets to resolve common UX failures inherent in legacy EHR systems, such as excessive clicking, copy-paste loops, and convoluted navigation hierarchies.[3] Components must include smart defaults, collapsible input sections to manage field density, and inline tagging mechanisms to expedite data entry.[3] Furthermore, the system must utilize a comprehensive design token architecture to manage typography, spacing, and a strict color palette.[3]

Compliance with the Web Content Accessibility Guidelines (WCAG) 2.1 Level AA is a non-negotiable requirement for all interfaces across the platform.[1] The UI must utilize high-contrast color schemes, scalable typography, and clear visual hierarchies to reduce the cognitive load on clinical staff and ensure usability for elderly or impaired patient populations accessing patient-facing portals.[5] For data-dense screens, such as the unified member profile, the design must aggregate information logically. Overloaded data blocks must be mitigated through consistent iconography, standardized data grouping, and the strategic use of negative space to guide the user's eye toward critical actionable items.[3]

Module 3.1: Care Delivery Interface Architecture

The Care Delivery module encompasses the end-to-end clinical workflow, integrating prospective patient scheduling, telehealth video consultations, clinical charting, and automated billing synchronization.[1] The UX must bridge the gap between administrative task management and focused clinical care, ensuring that providers spend their cognitive energy on the patient rather than wrestling with the interface.

The scheduling interface must support both prospective patient self-service and staff-assisted booking. The calendar component in Figma should utilize a grid system with dynamic slot generation based on provider availability.[1] To minimize no-shows, automated appointment reminders are critical. The UI should include an administrative configuration panel where staff can toggle SMS and email reminders at configurable intervals (e.g., 48 hours and 2 hours prior to the visit), with a corresponding read-only log embedded within the patient's record to verify message delivery.[1]

The telehealth interface must transcend a simple video feed. Treating video consultations merely as digital replicas of in-person visits often results in cold, transactional experiences that fail to build clinical trust.[9] The provider interface requires a sophisticated picture-in-picture (PiP) layout or a flexible split-screen design. This layout allows the physician to maintain eye contact with the patient while simultaneously reviewing the pre-visit chart and authoring action plans.[1] Pre-visit charts must be rendered in a side-panel component, displaying prior notes, action plans, and interaction histories in a condensed, highly scannable format accessible at least thirty minutes prior to the appointment.[1]

Clinical documentation requires charting interfaces that support rapid, structured data entry to minimize administrative burden. Components should include dynamic note templates (such as SOAP notes) that adapt to specific visit types, alongside rich-text input fields for narrative

documentation.[3] Action plan authoring requires structured components where providers can define tasks, assign owners, set due dates, and track statuses seamlessly.[1] To support the clinical staff supervision requirement, a dedicated "Review Queue" UI is necessary.[1] This dashboard must display pending notes from clinical staff, featuring distinct status badges (Draft, Submitted, Approved, Revision Requested) and one-click approval workflows to expedite provider sign-off.[1]

A critical UX challenge in care delivery is ensuring that clinical actions translate flawlessly into billable events without manual dual-entry. The UI must integrate time-based billing tracking components, such as a persistent, start/stop timer widget tied to the active patient encounter.[1] As the duration approaches specific Current Procedural Terminology (CPT) billing thresholds, the interface must deploy non-intrusive visual alerts, such as color shifts from cool to warm hues, indicating that a billing milestone has been reached.[1] Upon encounter closure, a modal must summarize the structured billing data (CPT codes, duration, provider ID) and display the real-time synchronization status with the Bridge/Candid API, utilizing success or failure toast notifications.[1]

Functional Requirement	**User Story ID**	**Figma Component Architecture & UI State Variations**
Prospective Scheduling	CD-01, CD-12	Interactive calendar grid utilizing Auto Layout. Variants for available, booked, and blocked slots. Modal overlays for staff-assisted booking with dynamic provider selection dropdowns.[1]
Appointment Reminders	CD-02	Toggle switch components for SMS/Email preferences. Timeline log items indicating reminder dispatch status (Sent, Delivered, Failed).[1]
Visit Summary & Feedback	CD-03, CD-04	Automated PDF generation templates. Post-visit survey components utilizing branched logic steppers

		with Likert scale radio buttons.[1]
Consent Management	CD-05	Integrated digital signature capture pad within the telehealth onboarding flow. Timestamped audit log table rows featuring locked state icons.[1]
Telehealth Visit Conduct	CD-06	Embedded WebRTC video container component. Variants for full-screen, split-screen, and picture-in-picture modes. Integrated pre-visit context side-panel.[1]
Consultation Notes	CD-07	Accordion components for historical note expansion. Skeleton loaders for pre-visit chart rendering to manage perceived latency.[1]
Action Plan Authoring	CD-08, CD-14	Dynamic form fields with rich text support. Version history side-sheet displaying author metadata and timestamp differentials.[1]
Staff Supervision Queue	CD-09, CD-15	Kanban-style review board or data table with status pill badges (Pending, Approved). Quick-action floating action buttons (FAB) for bulk sign-off.[1]
Billing & Time Tracking	CD-10, CD-17	Persistent global timer widget anchored to the viewport. Progress bar filling to indicate proximity

		to CPT time thresholds. Sync status toast notifications.[1]
Eligibility Check	CD-11	Real-time API query indicator (spinner). Status badges (In-Network, Out-of-Network, Verification Required) displayed within the scheduling flow.[1]
Action Plan Review	CD-13	Read-only checklist components. Status update dropdowns triggering automated notification toasts to the supervising provider.[1]
Patient Prioritization	CD-16	Sortable data table with dynamic priority scoring column. Color-coded badges mapping CPT time requirements to urgency levels.[1]
Task Management	CD-18	Unified task list with drag-and-drop prioritization. Filter chips for status, member, and date range. Overdue items flagged with high-contrast red typography.[1]
SDoH Assessment	CD-19	Mobile-responsive, step-by-step questionnaire components. Conditional logic revealing risk threshold alert banners upon form submission.[1]

Module 3.2: Case Management and Unified Member

Context

The Case Management module serves as the operational nucleus for member outreach, replacing fragmented CRM workflows with a cohesive, auditable system.[1] The primary objective of this module's UX design is to equip Case Managers (CMs) with comprehensive, actionable context immediately upon opening a patient file, completely eliminating the need for context-switching across external applications.

The core of this module is the Unified Member Context screen, often referred to as the Patient 360 dashboard. This interface must seamlessly aggregate clinical data, social determinants of health (SDoH) flags, communication histories, and active referrals into a single, cohesive dashboard.[1] To anchor the user's attention, a "Key Member Info" sticky header or persistent left-hand panel is essential. This panel must prominently display immutable and high-priority attributes: the member's Plan ID, primary language, ZIP code, preferred contact method, and calculated risk tier.[1] By keeping this information perpetually visible regardless of vertical scrolling, Case Managers are adequately prepared before initiating any outbound communication.

To present a unified communication history, the design must incorporate an Activity Feed component.[1] This feed chronologically aggregates SMS messages, emails, and embedded Five9 call recordings.[1] The UI design for this feed should utilize distinct, easily recognizable iconography for each channel, clear timestamps, and collapsible interaction notes to maintain a clean interface while offering deep contextual data on demand.[12] This chronological approach prevents narrative fragmentation, allowing a new Case Manager to instantly understand the trajectory of a member's engagement.

Case intake and SDoH referral workflows require highly optimized forms. Cases generated automatically from monthly claims files or eligibility updates must populate into a priority-ranked queue.[1] The Priority Engine UI must utilize a configurable scoring model, surfaced to the user via visual badges, ensuring that highest-need members are immediately apparent.[1] For SDoH referrals to third-party suppliers, the UI must feature a dynamic referral form linked to the supplier network directory, complete with a linear status tracker (Referred → Accepted → Fulfilled/Closed) to provide real-time visibility into service delivery.[1]

A critical component of modern case management involves mobile-first workflows for field staff. Community Health Workers (CHWs) and Canvassers operate in field environments where cellular connectivity is frequently unreliable or entirely absent.[13] Consequently, the mobile application design must adopt an aggressive "offline-first" UX strategy.[13] The UI must clearly communicate synchronization status without generating user anxiety or utilizing overly technical jargon. Essential design patterns include skeleton screens for loading states, determinate progress bars for active syncing, and clear visual indicators (such as a cloud icon with a slash) when the device is operating offline.[11] Data captured offline—such as Plan of Care electronic signatures, door-to-door visit dispositions, and secure photo captures of housing

documents—must be stored locally in an encrypted queue.[1] The UI must display a "Pending Sync" list in an outbox format, executing background synchronization automatically upon reconnection to a stable network, and resolving conflicts through predefined administrative logic.[16]

Functional Requirement	User Story ID	Figma Component Architecture & UI State Variations
Integrated Case Intake	CM-01	Auto-generated case cards appearing in a prioritized workload queue. Duplicate detection warning modals requiring manual override.[1]
Unified Member Context	CM-02, CM-03	Patient 360 dashboard featuring a sticky demographic panel, tabbed navigation for clinical vs. social data, and an aggregated interaction timeline.[1]
Case Surveys	CM-04	Modular survey builder UI. Front-end survey rendering with dynamic branch logic. E-signature canvas component for consent capture.[1]
SDoH Referrals	CM-05	Supplier directory search component with auto-complete. Linear progression stepper indicating referral lifecycle status.[1]
PCP Verification	CM-06	API-connected lookup search bar with debounce logic. Verified badge appended to the physician's

		name within the member profile.[1]
Appointment on Behalf	CM-07, CM-08	Integrated calendar modal allowing CMs to select member preferences (modality, time) via pill selectors and book external visits.[1]
Communication Feed	CM-09, CM-12	Chat bubble components for SMS. Embedded audio player UI for Five9 recordings. Chronological sorting with channel filter chips.[1]
SDoH Needs Feed	CM-10, RF-06	Cross-module notification center or dedicated feed panel. High-urgency items flagged with pulsing indicator animations.[1]
Funnel Management	CM-11	Managerial Kanban board visualizing member progression through engagement stages. Drag-and-drop cards with transition rate micro-charts.[1]
CHW Mobile Workflows	CM-13	Mobile-responsive native app UI. Touch-optimized large target areas. Plan of Care form with full-screen signature capture overlay.[1]
Canvasser Offline UI	CM-14	Persistent network status indicator bar. Local data cache queues represented as an "Outbox". Secure camera integration API

		hooks.[1]
Admin Logic Builder	CM-15, CM-20	No-code visual rule builder utilizing drag-and-drop logic blocks (IF/THEN parameters). Effective date scheduling date-pickers.[1]
Priority Engine	CM-16	Administrative scoring matrix table. Dynamic priority scoring badges rendered on member cards, updating via daily calculation triggers.[1]
Workload Management	CM-17	Dashboard of unassigned/overdue cases. Bulk selection checkboxes and reassignment dropdowns. Age indicators using color gradients.[1]
Ops / CM Reporting	CM-18	Real-time chart components (bar, line, pie) replacing Tableau. Configurable date range pickers and metric toggle switches.[1]
Sequential Handling	CM-19, SUP-08	Workflow dependency lock UI. Secondary cases rendered with a disabled state and a lock icon until the primary case status equals "Closed".[1]

Module 3.3: Contact Center Integration and Telephony UX

The Contact Center module seamlessly integrates telephony operations directly into the Case Management platform via Five9 and Twilio APIs.[1] The UX must prioritize absolute speed and contextual awareness, reducing the cognitive load on agents handling high volumes of inbound

and outbound calls. In a contact center environment, every click and every second of interface latency translates directly into increased operational costs and degraded patient experience.

A paramount feature of the modern contact center interface is the inbound "screen pop." When an inbound call arrives, the system utilizes Automatic Number Identification (ANI) to match the caller against the master patient database. The UI must render the corresponding member's full profile within an aggressive three-second Service Level Agreement (SLA) from the moment the call connects.[1] If multiple records match the incoming number or the caller is entirely unknown, the UI must gracefully fall back to a rapid manual search interface or a standardized "Create New Lead" modal.[1] The screen pop prevents agents from spending the critical first thirty seconds of a call requesting basic identifying information, thereby drastically reducing average handle time and establishing immediate, personalized rapport.[19]

For outbound campaigns, a highly efficient Power Dialer interface is required. The UI must present the next dial in the queue automatically upon the disposition of the previous call, removing the friction of manual dialing.[1] Crucial UI elements for this feature include a configurable delay timer displaying a countdown between calls, prominent Pause/Stop controls to grant agents necessary administrative breathing room, and mandatory disposition dropdowns that physically prevent the dialer from advancing until the outcome of the current call is accurately logged.[1]

During an active call, an interaction note-taking panel must automatically open alongside the member profile in a split-screen or persistent side-panel layout. This panel must support real-time, asynchronous saving to prevent data loss in the event of browser crashes. Furthermore, the active call UI must provide quick-access floating toolbars for mid-call actions, such as administering scripted surveys, sending follow-up SMS messages via Twilio, or initiating warm transfers to external supplier partners with full contextual data handoff.[1] Supervisors require robust list management tools, utilizing multi-select data tables and dynamic filtering by plan type, language, and clinical priority to generate targeted campaigns seamlessly.[1]

Functional Requirement	**User Story ID**	**Figma Component Architecture & UI State Variations**
External Dialing	CC-01	Click-to-dial text links applied globally to all phone number fields. Automatic invocation of the active call logging panel.[1]
Power Dialing	CC-02	Persistent dialer control bar (Play, Pause, Skip) anchored

		to the viewport. Automated queue progression with mandatory call disposition blockers.[1]
Inbound Screen Pop	CC-03	Instant-load profile overlay triggered by CTI webhook events. Designed to render critical data above the fold within 3 seconds.[1]
Active Call Interface	CC-04	Split-pane view showing member context alongside an active interaction log. Auto-save status indicators (Cloud checkmark icon).[1]
Warm Transfers & Surveys	CC-05	In-call quick-action toolbar. Survey stepper embedded within the call panel. Transfer modal with internal/external directory search.[1]
Dial List Management	CC-06	Supervisor dashboard with dynamic query filters. Progress bars indicating list penetration and agent completion rates.[1]
Ad-hoc Outreach	CC-07	Distinct visual styling for "Lead" records versus "Member" records (e.g., differentiated header colors). Conversion workflow button.[1]
Consultation Scheduling	CC-08	Scheduling modal accessible directly from the lead record. Provider calendar integration with available slot selection.[1]

Module 3.4 & 3.8.1: Community Events and Virtual Platform Architecture

The Events module transitions community engagement from unstructured, ad-hoc processes into highly structured, measurable operational workflows.[1] Community Leads require sophisticated tools to manage both physical, in-person gatherings and large-scale virtual townhalls seamlessly from within the primary CMS application.

To facilitate event discovery, the platform must feature an integrated Event Finder component, allowing both members via the self-serve portal and staff via the CMS to search for activities utilizing location radii and interest-based categorical filters.[1] For event organizers, the Roster Management UI must present a comprehensive data table. A unique requirement here is the integration of predictive analytics; the roster UI must display a predictive attendance score based on the member's historical engagement, location, and expressed interests.[1] This table must support bulk-select checkboxes, enabling Community Leads to execute rapid RSVP status updates or dispatch segmented bulk SMS communications directly to the attendee list without leaving the view.[1]

To replace reliance on disparate video conferencing tools, a deeply integrated Virtual Platform is required for hosting townhalls and teleconferences.[1] The moderator UI must provide comprehensive control over the session environment. Essential Figma components include a live, dynamically updating attendee list with precise join/leave timestamps, a visual hand-raise queue that automatically prioritizes speakers in chronological order, and global administrative controls for muting all participants or banning disruptive attendees.[1] The interface must also include an in-session rich-text notes panel. Following the conclusion of the event, the system must automatically append the attendance data and session notes to each participating member's interaction timeline. This automated synchronization generates engagement metrics that populate managerial reporting dashboards, displaying attendance rates, RSVP-to-actual conversion funnels, and engagement trends segmented by event modality.[1]

Functional Requirement	User Story ID	Figma Component Architecture & UI State Variations
Roster & RSVP Management	EV-01	Advanced data grid with inline editing for RSVP status (Yes/No/Maybe). Predictive attendance scoring indicators (High/Med/Low badges).[1]

Bulk Communications	EV-02	Segmentation selection checklist. WYSIWYG SMS message composer with real-time character count and dynamic personalization tags (e.g., {{FirstName}}).[1]
Event Reporting	EV-03, VP-08	Analytical dashboard featuring bar charts for attendance rates and line graphs for engagement trends over time. CSV export buttons.[1]
Virtual Event Hosting	VP-01, VP-07	Embedded WebRTC video interface. Recording initiation toggle with glowing red recording indicator. Secure cloud storage link generation.[1]
Attendance Tracking	VP-02, VP-06	Live attendee list panel. Automated reconciliation logic triggering interaction record creation upon session termination.[1]
Moderator Controls	VP-03, VP-04	Command sidebar featuring mute all toggles, individual participant action menus (three-dot icon), and sequential hand-raise queue lists.[1]
Session Notes	VP-05	In-session rich text editor panel adjacent to the video feed. Auto-save functionality pushing notes to the master event record on close.[1]

Member Event Finder	EV-05	Search interface with geospatial and categorical filter chips. Event detail cards featuring prominent RSVP Call-to-Action (CTA) buttons.[1]
Self-Serve Portal	EV-06	Mobile-optimized member portal UI. List view of upcoming registered activities. One-click RSVP modification controls updating the backend roster.[1]

Module 3.5: Communication, Messaging, and Marketing Automation

This module governs all outbound and inbound communication channels—SMS, MMS, email, and automated marketing campaigns—ensuring that every touchpoint is recorded, compliant, and accessible within the unified member timeline.[1]

The SMS and MMS viewing interface must emulate modern consumer chat applications to ensure rapid staff adoption. The UI should utilize a familiar back-and-forth chat bubble layout, distinctively color-coded to differentiate between staff messages, automated system notifications, and member replies.[1] MMS images must be rendered inline as thumbnails, with a lightbox overlay for full-resolution viewing. Crucially, all communications must be automatically appended to the member's core interaction log, eliminating the need for manual transcription.

To support automated outreach, the CMS requires a bidirectional synchronization interface with Salesforce Marketing Cloud (or equivalent tools like Klaviyo). The Admin UI must allow technical staff to define mapping rules and trigger conditions (e.g., reactive event-based triggers, or time-based schedules).[1] Furthermore, the platform must capture and enforce rigorous member communication preferences. The profile UI must include a dedicated preferences panel storing the preferred channel, optimal contact days and times, language requirements, and strict opt-in/opt-out toggles to maintain compliance with anti-spam regulations.[1]

Functional Requirement	**User Story ID**	**Figma Component Architecture & UI State Variations**

SMS / MMS Storage & UI	MSG-01	Chat interface with left/right aligned message bubbles. Inline image rendering for MMS. Infinite scroll pagination for historical message retrieval.[1]
Marketing Automations	MSG-02	Administrative mapping dashboard. Toggle switches for campaign triggers. Visual indicators denoting successful API synchronization with external marketing platforms.[1]
Communication Preferences	MSG-03	Dedicated profile section utilizing radio buttons for channel preference, multi-select dropdowns for day/time, and prominent opt-out warning banners.[1]
Bulk SMS Engine	MSG-04	Shared component utilized across Case Management and Events. Includes audience preview count, message drafting area, and pre-flight delivery estimation.[1]
Resource Finder Cases	MSG-05, RF-01	Webhook listener indicator. Auto-generated case cards populated with RF payload data (need type, timestamp) and SLA timer icons.[1]

Module 3.6: Supplier Network Extensibility and Partner Workflows

The platform must facilitate seamless, secure collaboration with external partners—specifically MediCircle for pharmacy coordination and Upside for housing and field support—without compromising the core platform's data integrity or exposing unauthorized PHI.[1] The UX must provide tailored supplier views that surface only the data relevant to the partner's specific operational mandate.

For the MediCircle integration, the dashboard must feature a highly specialized Multi-Rx Bundle Management component.[1] Because patients often require multiple prescriptions synchronized for a single delivery, this UI should display individual prescription statuses nested within a broader patient bundle container. The system must trigger clear visual alerts (e.g., a high-contrast badge stating "Ready in 3 Days") when a bundle reaches fulfillment parameters.[1] Furthermore, to streamline operations, a "Consent Bypass" indicator must be prominently displayed on the supplier's member view, instantly informing pharmacists if valid HIPAA consent is already on file, thereby allowing them to bypass redundant consent scripts.[1]

For the Upside integration, the interface must support bidirectional shadow tracking. This involves a synced status stepper component visible on the Wider Circle member profile (e.g., "Assigned" → "Visit Scheduled" → "Completed"), providing internal staff with real-time visibility into the partner's workflow without requiring direct access to the partner's external systems.[1] Upside personnel require a specialized team assignment view to manage their caseloads, featuring inactivity alerts that notify supervisors if outreach SLAs are breached.[1]

Functional Requirement	**User Story ID**	**Figma Component Architecture & UI State Variations**
Prescription Alerts	SUP-01	Configurable threshold input fields in admin settings. Alert banners appearing in the MediCircle supplier view with quick-action SMS notification buttons.[1]
Dial & Interaction Logging	SUP-02	Standardized interaction modal capturing timestamp, outcome dropdown, and contact type radio buttons. Read-only log visible to WC staff.[1]

Multi-RX Bundle UI	SUP-03	Nested accordion tables. Parent row displays overall bundle status; expanding reveals individual Rx line items with specific statuses.[1]
Consent Bypass	SUP-04	Prominent green "Consent on File" badge located in the member header. Conditional rendering logic hiding the consent script workflow if present.[1]
Upside Task Notifications	SUP-05	In-platform notification bell icon with unread count badge. Flyout panel displaying task assignments and status change alerts.[1]
Data Verification	SUP-06	Automated reconciliation dashboard. Discrepancies (Name, DOB) flagged with warning icons requiring manual admin review and resolution selection.[1]
Shadow Tracking	SUP-07	Read-only progression tracker component (horizontal stepper) mirrored from external system API statuses, visible on the master member profile.[1]
Sequential Enforcement	SUP-08	Workflow dependency lock UI. Secondary cases rendered with a disabled, grayed-out state and a lock icon until the primary case reaches terminal status.[1]

Team Assignment	SUP-09	Drag-and-drop assignment board for Upside supervisors. Inactivity alert icons appearing on stale member cards based on configurable SLA timers.[1]
Location Discovery	SUP-10	Geospatial tagging input fields with address type flags (Permanent/Temporary). Interactive map component plotting known location history.[1]
Document Relay	SUP-11	Drag-and-drop document upload zones for field staff. Automated routing logic creating a notification task for the assigned internal Case Manager.[1]
Automated Outcomes	SUP-12	Configurable report generation template. Scheduling interface for automated email or SFTP delivery of aggregated visit summaries.[1]
Supplier Referral Tracking	SUP-13	Centralized referral directory search within the case view. Referral creation form capturing service type and date, linked to the status tracker.[1]

Module 3.7: Reporting, Data Ingestion, and Administrative Controls

Replacing reliance on external visualization tools like Tableau requires embedding highly performant, real-time operational dashboards directly within the CMS application layer.[1] Furthermore, system administrators require robust, no-code interfaces to manage complex

business logic without constant engineering intervention.

Following best practices for healthcare dashboards, the UI should adopt an "exception-based" design philosophy.[21] Rather than overwhelming operations managers with dense tables of nominal data, the dashboard should explicitly highlight anomalies, operational bottlenecks, and threshold breaches. Visualizations should utilize gauge charts, "Red/Yellow/Green" status lights, and directional trend arrows to instantly communicate whether Key Performance Indicators (KPIs)—such as case throughput, billing pipelines, and CM productivity—are meeting established benchmarks.[21]

To support data engineering and billing workflows, the platform must feature an Interaction Validation queue. This UI highlights records missing mandatory compliance fields (e.g., missing CPT codes or absent provider signatures), physically blocking them from the billing data export payload until resolved.[1] The administrative rule builders must utilize an intuitive drag-and-drop interface with clear boolean logic operators (AND/OR). This allows non-technical system administrators to construct member segmentation rules, define automated assignment routing based on geography or language, and replicate complex legacy Salesforce automations visually.[1]

Functional Requirement	**User Story ID**	**Figma Component Architecture & UI State Variations**
Assignment Rules	DA-01	Visual logic builder mapping geographic, linguistic, or plan-type variables to user groups. Manual override toggles within the member profile.[1]
Interaction Validation	DA-02	Review queue data table highlighting invalid records. Missing fields highlighted in red. Bulk validation action buttons.[1]
Data Ingestion UI	DA-03	Upload interface for claims files. Progress bars for ingestion. Error reporting log displaying row-level validation failures.[1]

Billing Data Export	DA-04	Export configuration panel allowing selection of schema format (CSV/JSON), date ranges, and programs. Scheduled run history table.[1]
Member Segmentation	DA-05	Dynamic query builder interface utilizing multi-select dropdowns for clinical risk and engagement status. Auto-refreshing cohort count display.[1]
Member Journey View	DA-06	Visual timeline map illustrating journey stages (Available → Engaged). Next-best-action recommendation cards surfaced by algorithmic logic.[1]
Salesforce Migration	DA-07	Administrative mapping tables to track parity between legacy Salesforce flows and new platform automations during the transition phase.[1]
Billable Interaction Rules	DA-08	Configuration matrix defining required CPT codes, time thresholds, and documentation standards per programmatic workflow.[1]
Operational Dashboards	DA-09	Role-specific layouts featuring exception-based gauge charts, trendlines, and funnel visualizations updating at ≤ 5-minute

		intervals.[1]
Info Enrichment	DA-10	API connection status indicators for Neustar integration. Enriched data fields displayed with a distinct subtle background color to denote external origin.[1]

Module 3.8: Application Surfaces (Resource Finder and Portals)

The platform must integrate seamlessly with external application surfaces, including the Member App, Self Serve Portal, and the existing Resource Finder (RF) tool used to locate community support services (e.g., food pantries, housing assistance).[1]

The UX strategy for the Resource Finder integration involves eliminating repetitive data entry, a common source of frustration for both members and staff. When a user transitions from the CMS or the patient portal into the RF environment, a secure token-based handoff must pre-populate the member's context—such as ZIP code, primary language, and existing SDoH flags—directly into the RF search interface.[1] This ensures that resource results are instantly filtered for relevance.

Conversely, the data flow must be bidirectional. Any resource needs identified or referrals initiated within the RF must trigger a webhook back to the core platform. The CMS UI must instantly translate this payload into a prioritized case card, starting an SLA timer, and appending the RF browsing activity to the member's unified interaction timeline to maintain comprehensive context.[1]

Functional Requirement	**User Story ID**	**Figma Component Architecture & UI State Variations**
RF Case Auto-Creation	RF-01	Webhook listener triggering auto-generated case cards. Follow-up timing badges (e.g., "Next Day", "3 Days") based on need urgency.[1]
Escalation Routing	RF-02	Automated routing logic

		pushing RF escalations to specific CM queues. Supervisor alert notifications triggered if SLA timers expire.[1]
Context Pre-Population	RF-03	SSO handoff mechanism. Hidden form fields transferring member ZIP, language, and SDoH flags to seamlessly filter RF search results.[1]
Interaction Logging	RF-04	Timeline log entries recording RF session start/end times, resources viewed, and source identification (Self-serve vs. Staff-assisted).[1]
Referral Status Tracking	RF-05	Webhook integration displaying RF referral outcomes (Confirmed/Declined) within the CMS case file. Manual status override dropdowns.[1]
SDoH Feed Integration	RF-06	Real-time surfacing of RF-identified needs into the global SDoH Needs Feed. High-urgency flags driving immediate CM attention.[1]
Feedback Follow-Up	RF-07	Automated generation of a "Feedback Case" type upon referral fulfillment. Embedded survey components to capture resource efficacy.[1]

Module 3.9: Ask Claire Integration and Compliance

(RBAC)

The integration of Ask Claire—a fully owned Medicare brokerage subsidiary—presents the most complex architectural and UX challenge within this engagement. Because Ask Claire operates under strict Centers for Medicare & Medicaid Services (CMS) marketing and communication regulations (42 CFR Part 422/423) and Wider Circle functions as a clinical provider, absolute data isolation is legally mandated.[1] The UX must enforce these boundaries without completely shattering the utility of a unified system.

To prevent anti-steering violations, the platform architecture must enforce strict multi-tenant Role-Based Access Control (RBAC) patterns.[24] The UI must clearly delineate entity boundaries at the presentation layer. An Ask Claire agent must never see clinical notes, SDoH assessments, or action plans, while a Wider Circle clinician must be entirely shielded from brokerage commissions, plan selection histories, and enrollment workflows.[1] In Phase 3, when both entities operate within the single unified platform, the Figma design must utilize distinct visual cues—such as varied global color themes, persistent role badges in the navigation bar, or workspace watermarks—to constantly remind users of their current compliance context and access level.[26]

Data flowing across the compliance boundary (for example, a duals member transitioning from an Ask Claire enrollment directly into a Wider Circle care management program) must be backed by explicit, timestamped patient consent.[1] The handoff UI must include a mandatory consent acknowledgment checkbox and capture the originating system's payload. To prevent duplicate, harassing outreach to the same member, the UI must display a shared, read-only interaction summary. This panel indicates the date and basic disposition of recent cross-entity contacts, while aggressively masking the specific agent identities and redacting sensitive call transcripts to maintain compliance.[1]

Functional Requirement	User Story ID	Figma Component Architecture & UI State Variations
Member Referral Intake	AC-01	Read-only referral card generated from external payload containing demographic data and alignment date. Assignment notification toasts.[1]
Duplicate Outreach Block	AC-02	Warning banner or modal interrupting dial attempts if

		the member was contacted by the sister entity within a configurable suppression window.[1]
Duals Eligibility Sharing	AC-03	Read-only alignment status fields on the member profile sourced from the AC data feed. Masked agent ID fields to protect privacy.[1]
Handoff Workflow	AC-04	Handoff form capturing reason and recommended program. Generates a priority task in the receiving entity's CMS queue.[1]
Consent-Backed Transfer	AC-05	Mandatory digital signature or consent checkbox prior to payload transmission. Rejected transfers flagged in an administrative integration log.[1]
Communication Visibility	AC-06	Shared interaction summary panel showing basic contact metadata (Date, Channel, Disposition) while restricting access to full transcripts.[1]
Cross-Entity Dashboard	AC-07, AC-08	Executive-level aggregate dashboards stripped of all individual Protected Health Information (PHI), showing funnel conversions across both entities.[1]
Unified Platform (RBAC)	AC-09, AC-10	Tenant-scoped permission architecture. Strict UI visibility toggles hiding clinical/brokerage modules

		based on authenticated role context.[1]
Cross-Entity Audit Log	AC-11	Immutable, highly searchable data table logging user ID, entity, data accessed, timestamp, and purpose code for rapid CMS audit reporting.[1]
AC Outreach Tracking	AC-12	Ask Claire-specific interaction log capturing call outcomes and follow-up tasks, isolated within the AC entity boundary.[1]

Module 3.10: Shared Medical Appointments (SMA) UX Architecture (Future Scope)

While explicitly deferred from the Phase 1 and Phase 2 implementation, the architectural groundwork for Shared Medical Appointments (Module 3.10) must be established within the Figma design system immediately. Failing to account for this complex workflow now will result in costly, foundational refactoring in the near future. SMAs represent a highly efficient care delivery model where a cohort of patients sharing a common chronic condition receives high-yield education together, followed by individualized, private check-ins with a billing provider.[1]

Group Event Management and Patient Rostering

The foundation of the SMA module is the creation of a "Master Group Record." The UI must allow clinical or administrative staff to construct cohorts, typically ranging from 8 to 12 patients, to ensure financial viability and optimal group dynamics.[29] The rostering interface must cross-reference patient eligibility, utilizing advanced filtering to ensure all participants share the targeted diagnosis (e.g., Type 2 Diabetes) and have explicitly consented to the unique privacy dynamics of a group setting.[28] For virtual SMAs, the integrated video interface must support a gallery view accommodating all participants alongside a multidisciplinary care team (e.g., Provider, Health Coach, Medical Assistant), with robust moderator controls to manage discussion flow.[2]

Dual-Pane Group Charting and Clinical Documentation

The most significant UX challenge in executing an SMA is the documentation burden. The

provider must document the encounter to satisfy the strict "Medical Decision Making" criteria required for standard Evaluation and Management (E/M) CPT codes (typically 99213 or 99214) for *each individual patient* participating in the session.[2]

To facilitate this efficiently without forcing the provider to open and close twelve separate patient charts, the UI must employ an innovative split-screen or dual-pane charting layout.

- **The Global Pane (Left):** This section captures the generalized group education notes, the specific curriculum discussed, and the total time spent in the group setting. This data is automatically replicated and appended to all participant records simultaneously.[29]
- **The Individual Pane (Right):** This is a rapid-switching interface, essentially a carousel of patient charts. It allows the provider to quickly toggle between patients to document their private check-in, record vital signs, note individual medication adjustments, and author personalized action plans without ever exposing one patient's PHI to another's record on the screen.[29]

Batch Billing Validation Workflows

Because there are no dedicated, national CPT codes specifically designed for SMAs, the billing system must treat the group event as a batch of individual follow-up visits.[2] This requires a specialized billing validation interface. The billing UI must aggregate the outputs from the individual documentation panes and present them in a validation matrix. This matrix verifies the presence of the required 99213/99214 criteria per patient (e.g., stable chronic illness, complexity of data reviewed). Only when all individual patient encounters within the SMA are validated does the system enable the "Submit Batch" button, executing a bulk synchronization payload to the RCM system (Candid/Bridge) upon the final closure of the Master Group Record.[1]

SMA Functional Requirement	Proposed Figma UI Component & Design Pattern	Strategic UX Objective & Justification
Cohort Assembly & Rostering	Multi-select data grid with diagnostic filtering criteria. Integrated consent verification badges indicating HIPAA compliance for group settings.	Ensure only eligible, established patients are booked into the group session, fulfilling baseline billing prerequisites.[28]
Privacy & Consent	Digital consent form modals specifically detailing	Maintain strict HIPAA compliance within a

Management	the sharing of health information in a group context. E-signature capture.	communal environment, protecting both the patient and the provider.[28]
Split-Pane Group Charting	Persistent left pane for common group education notes; right pane featuring a tabbed carousel of individual patient charts.	Maximize provider efficiency by writing shared educational notes once, while preserving necessary individual clinical fidelity.[29]
Batch Billing Validation	Pre-submission review dashboard (matrix table) verifying individual E/M criteria (99213/99214) for all attendees simultaneously.	Prevent claim denials by ensuring each individual patient encounter within the SMA meets stringent RCM standards before API sync.[2]
Virtual SMA Hosting	Embedded WebRTC gallery view interface. Moderator controls tailored for group participation (round-robin speaking queues, hand raises).	Facilitate engaging, organized group discussions that mimic the support dynamics of in-person Shared Medical Appointments.[2]

Synthesis and Strategic Outlook

The architectural design of the Unified Internal Platform represents a fundamental paradigm shift for community health operational capabilities. By systematically migrating away from disjointed legacy systems into a cohesive, component-driven framework designed natively in Figma, the organization can dramatically reduce clinical friction, eliminate double-entry errors, and lower administrative overhead.

The success of this platform hinges on the rigorous application of exception-based design principles to prevent data fatigue, the implementation of robust offline-first mobile workflows for field operatives, and the strict enforcement of role-based access controls to maintain compliance boundaries—particularly regarding the Ask Claire integration. Furthermore, by embedding the foundational UI patterns for complex future workflows, such as the dual-pane charting and batch billing validation required for Shared Medical Appointments, directly into the design system today, the platform is positioned to scale elegantly. This strategic foresight ensures the system can support expanding patient populations and innovative care delivery models without requiring costly, foundational redesigns in the future.

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